
HAVEN PAEDIATRIC CENTRE

Welcome to Haven, Again

Your onboarding and reorientation pack

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FOR
Everyone at Haven Paediatric Centre

FROM
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ABOUT
What we are building, what it means for you, and
what happens next

01 Why everybody is getting this

Haven is a little over two years old. In that time this team has built something that is genuinely hard to build: a hospital where people care about the work. We know that is true because you told us, anonymously, in a survey where you could have said anything at all. Sixteen of seventeen of you rated the safety of care here as very good or excellent.

What has not been built yet is the scaffolding around that. Not much is written down. The forms were designed for a far smaller hospital than the one you are now running. Most of what works here works because somebody remembered, or stayed late, or knew who to ask.

So we are starting everybody again, at the same time. Every single person at Haven, from the ward to the front desk to the pharmacy, is being re-onboarded over the next few months. You will get a written description of your job, agreed measures, written standards for the work, a colleague paired with you, and a place on a development assessment.

WHO THIS APPLIES TO	Everybody. All nineteen of us.
WHY EVERYBODY	Because this is about the hospital, not about any one person
WHAT IT IS NOT	A response to anything anyone did
HOW LONG	About three months to put in place

That last line matters, so it is worth saying plainly. Nobody is being singled out. Nothing here is a reaction to any individual's work. We are starting everybody together precisely so that nobody has to wonder whether this is about them.

02 What we are trying to achieve

A fuller hospital that is still a safe one. The cots in use, nothing running out, nothing missed, and nobody finding out about a problem too late to fix it.

The hospital gets busier from November to February. That is every year, and it is the most demanding stretch of your working year. Everything we are putting in place now exists so that you walk into that season with the stock, the rota and the standards already sorted, instead of discovering in December that they are not.

The goal is not for you to work harder in January. It is for the system to carry weight in January that you have been carrying yourselves.

03 How we will all see how we are doing

You should not have to guess whether things are getting better. So there will be a scoreboard, put up where people work and updated every week. Six things, because a scoreboard nobody can hold in their head is just a report.

What we watch	Why it is on the list
Beds and cots in use	A cot ready within the hour is what turns a referral into an admission
Discharges finished before noon	Decided at the morning huddle. This one is entirely yours
Stockouts this month	The target is zero. Reaching for something that is not there is a clinical event
Near misses reported, and how many are closed out	Reports going up is a good month. See section 6
Standards written and in use	Only goes up when a team has signed off its own
Rota published four weeks ahead	A streak, not a percentage. It breaks visibly

None of these is a league table and none of them names anybody. They are about the hospital and about each unit, measured against last month.

04 What good looks like in your job

Right now, almost nothing at Haven is written down. That means people are sometimes corrected for doing a task the way they were taught it, which is not fair to anybody.

Over the next three months everyone gets three things on one page: what your job is for, the handful of things it is accountable for, and the three to five measures you are actually held to. Never more than five. Never a measure you cannot influence.

Alongside that, each team writes the standards for its own area. Not us writing them and handing them over. The first question in every one of those sessions will be the same.

Show me how you do this now.

Most of the time what you already do is sound and simply unwritten, and the standard will be a recording of it rather than a change to it. Where something does need to change, you will be in the room when it is decided and you will know why.

The point of all of it is one sentence: the care a child gets here should not depend on who happens to be on shift that day.

05 How we work here, day to day

A few habits go in over the next few weeks. They are small and they are not negotiable, because they are what holds everything else up.

The habit	What it is
The morning huddle	Fifteen minutes, standing, every working morning. What happened overnight, the plan for each patient, who goes home today, and what will stop us
Handover	Same shape every time, written down, with a name and a time against it. The next person has to be able to trust it
The rota, four weeks ahead	Published a month out and kept that way, so you can plan your life
Stock	Proper reorder levels on the things that move, so the shelf is not empty when you reach for it
Your buddy	Fifteen minutes a week, on shift, not after it. See section 7

If a habit on that list turns out to be impossible in practice, that is a problem with how we set it up, not a failing on your part, and we want to hear about it rather than watch it quietly stop.

06 Speaking up

This is the most important section in this pack.

When something nearly goes wrong and you catch it, we want to know. Not so that anybody gets into trouble. A near miss is the cheapest lesson a hospital ever gets, and the hospitals that learn from them are the ones that stay safe when they get busy.

You can report one with your name on it or without. There is a form on the staff page and it takes a minute.

In the best hospitals studied anywhere in the world, reports of things going wrong go up, not down. That is not because those hospitals are worse. It is because people in them feel able to say what actually happened.

So when the number of near misses reported here rises, we will treat that as a good month and we will say so out loud. What we will look for in every single one is the system cause: a standard that did not exist, a supply that was not there, a handover that did not happen, a rota that left somebody doing two jobs. Those are ours to fix.

If reporting something ever costs you anything at all, that is a failure on our part and we want to hear about that too, directly.

There is a second form on the staff page for anything that is simply broken or wastes your time. One line is enough. You know where this hospital leaks time and effort far better than anyone who arrives with a clipboard.

07 Your buddy

Everybody will be paired with a colleague, and the pairings will be put up where everyone can see them.

The pair meets for about fifteen minutes a week, during a shift rather than after one. You walk each other through one thing from whatever standard is currently being written, and each of you names one near miss or one thing that wasted your time that week. One of you writes it down, taking turns.

A few rules worth knowing. You will usually be paired across shifts, so the pair gives you a line to a part of the hospital the rota does not already connect you to. Sometimes you will be paired across jobs, because a nurse and someone from the front desk understanding each other's constraints fixes more friction than either of them being trained again. You will never be paired with the person who writes your appraisal. And pairs rotate every few months, so this ends up as a network rather than a handful of fixed friendships.

08 Your assessment

Everyone at Haven is being given a place on a leadership and development assessment. Everyone, not a selected few.

It takes about an hour. It looks at how you tend to work, what you value, how you read and handle emotion, the move from doing the work to leading it, how you operate in a team, and how you see the culture around you. Nobody passes and nobody fails.

WHO SEES YOUR RESULTS

You. Only you.

WHO DOES NOT

Haven, the owners, your manager, anyone else

WHAT WE SEE

The pattern across the whole team, with no names

WHAT IT IS USED FOR

Building the training and support around what people actually need

That confidentiality is not a courtesy we are extending and could withdraw. It is how the assessment works everywhere we use it, and it holds here.

09 What happens over the next three months

Roughly when	What you will see
Now	This pack, the staff page, and your assessment invitation
Weeks 1 to 4	The morning huddle starts. The licence and record housekeeping gets done. The scoreboard goes up
Weeks 3 to 8	Standards written, one team at a time. Buddy pairs announced
Weeks 5 to 10	Job descriptions and measures agreed, one conversation per person
Through all of it	A short message from us most weeks, usually one question, usually one tap to answer

About those weekly messages. Part of what we are building is simply the habit of getting something from us and answering it, because none of the rest works if messages go unread. They will be short, most of them will take one tap, and we will publish back how many people answered. When something changes because of your answers, we will tell you what changed. When little changes, we will tell you that too.

10 Who to go to

For anything about your shift, your rota or your work day, your line manager first, as now.

For anything clinical, the Chief Medical Director, as now. None of this changes who is responsible for clinical decisions.

For anything about this programme, anything you think is wrong with it, or anything you would rather say to somebody from outside the hospital, us. We will be in the building and you can stop us in a corridor, or use the forms on the staff page, which can be anonymous.

One last thing. You built the part of this hospital that cannot be bought. We are here to build the part that can, so that what you built survives Haven getting bigger.

Tell us what is broken.

Dr Debo Odulana

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